

SIG 1**Clinical Focus**

Translanguaging in AACtion: An Expansive Approach to Multimodal Communication With Augmentative and Alternative Communication Users

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ABSTRACT

Purpose: The purpose of this framework is to challenge the deficit perspectives toward multimodal communication and to encourage speech therapists and classroom teachers to apply educational justice and equity practices with modally marginalized individuals in education and clinical practice.

Method: We summarize contemporary challenges in augmentative and alternative communication (AAC) as a practice area and integrate translanguaging and crip linguistics literature to propose a novel framework: Translanguaging in AACtion.

Results: The authors propose the Translanguaging in AACtion framework to provide a praxis for engaging with and supporting all language users in and beyond the classroom by rethinking service delivery, supporting reciprocal engagement, and letting students language. The Translanguaging in AACtion framework guides speech therapists toward supporting disabled nonspeaking individuals to use their full-bodied semiotics (modes, languages, and dialects) in every setting, with every person, in every situation.

Conclusions: This work highlights the languaging practices of (dis)abled, non-speaking language users. Next steps are suggested to support speech therapists and classroom teachers with rethinking their praxis, with a focus on AACtion.

How you language is beautiful. Don't let anyone tell you your languaging is wrong. Your languaging is the story of your life.

—Dr. Jon Henner

Our languaging practices and experiences share the stories of our identities, who we are, and who we want to be. When those practices are restricted and pathologized, our language use becomes oppressed and inauthentic. The definition of *language* is “the principal method of human

communication, consisting of words used in a structured and conventional way and conveyed by speech, writing, or gesture” and “a system of communication used by a particular country or community” (Oxford Languages, n.d., paras. 1–2). Despite this definition, the discussion of “language” is often narrowed to primarily focus on spoken language, leading to the pathologization of nonspeaking languages and language systems. Additionally, a significant focus of spoken language is on those who have “good” language skills versus those who have “bad” language skills. People who are seen as having “good language skills” are seen as being “intelligent,” while people deemed as having “bad language skills” are over critiqued, misunderstood, and underestimated in their language and cognitive abilities. Thus, when a student’s language performance is critiqued in this binary and they are said to have

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“bad language skills,” they are promptly referred to the school speech therapist to receive therapeutic intervention to improve their language skills to be more like the majority. Ultimately, how students language helps shape their educational, social, and recreational experiences, and this extends to all students, including nonspeaking, disabled students.

The early part of the first author’s (Karina’s) career as a speech therapist began in a public high school supporting students with complex communication needs. After her second year of practice, Karina began to observe the same attitudes and patterns around educating and supporting racially, linguistically, and modally minoritized students like the one presented in the case study below. A reoccurring theme was one that decentered the families and students as experts on how they used and engaged with language. Instead, the classroom teacher and speech therapist were centered as the sole experts prescribing a plan for how they will learn to use and engage with language. The work of collaboration between all key stakeholders (e.g., parents, caregivers, family members, classroom teachers, speech therapists, community members) to support the language practices of multimodal language users was not happening. The following case study describes a fictitious student called “Michael.” Michael’s story is a composite of real observations and experiences from years of working with young people just like him and is used to illustrate the argument for a novel, intersectional framework that supports students like him.

Michael is a 14-year-old, Black, autistic student attending an American public high school. He lives with his parents and siblings in a single-family home in the city, and his family and community primarily use African American English (AAE) in their daily conversations. Before he entered grade school, he was diagnosed with an intellectual disability by a psychologist utilizing the criteria outlined in the *Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition* (American Psychiatric Association, 2013). An Individualized Education Plan (IEP) was recommended when he was 3 years old and upon his entrance into the public school system. He receives specialized academic instruction in his special day class and participates in speech therapy services at school. According to his classroom teacher and speech therapist, he is nonspeaking and has “minimal language skills.” Michael is a multimodal communicator.

From the time that Michael was a toddler, he and his family have used a combination of gestures, signs, actions, pointing, leading, pictures, graphic symbols, game systems, videos, eye gaze, television shows, menus, verbal approximations, and a series of yes/no questions to communicate. When he was in elementary school, they also introduced a communication application on a tablet a few

different times, but the parents did not feel skilled in its implementation, and it was not being used in the classroom, nor supported at school.

Michael’s way of communicating was developed by him and his family, and it has worked for many years. However, this form of languaging was only intermittently being supported outside the home. Instead, with the revolving door of classroom teachers and speech therapists, every few years the classroom teacher and speech therapist would revisit prioritizing working on developing his verbal speech and write goals to improve his articulation and intelligibility. These goals were also supplemented with general communication goals using a “total communication approach” (Jones, 2000). Each year, most of these goals were considered “unmet” as the speech therapist reported minimal progress on his articulation and intelligibility. The speech therapist cited that he would not participate in therapeutic activities and often seemed frustrated, which resulted in “negative” behaviors in the classroom. The educational team all agreed and reported that he was frustrated because he “did not have a way to communicate.” His classroom teachers and speech therapists viewed him as “languageless” (which will be discussed more later in this clinical focus article). Michael and his family were referred to the speech therapist–augmentative and alternative communication specialist (AAC-SLP; AAC-SLP is used to refer to speech therapists who specialize in AAC) by his school district after complaints were lodged by the family related to Michael’s educational progress at school. The family felt as though they were being unheard, and the continuity of support for Michael’s language experiences was not being well supported at school.

Michael is not a unique case. Time and time again, students just like Michael, disabled and nonspeaking, are underserved and undersupported. Even though the students are provided speech and language therapy services at school, the traditional service delivery model (e.g., pull out services) and the goals written in IEPs do not consistently support students’ multimodal language development. Neither do they support the classroom educational team with teaching and sharing in language practices that the student can use in and beyond the classroom.

Traditional service delivery models for augmentative and alternative communication (AAC) users are challenging because students do not typically get the opportunity, through daily educational experiences and assessment practices, to access their full linguistic repertoire to show what they know (Tönsing & Soto, 2020). When children cannot or do not perform in ways that assessments can accurately measure their language, they are seen as deficient, and their skills are diminished. Even when dynamic assessment procedures are used, there is room for misdiagnosis and

mischaracterization of their knowledge and skills. While dynamic assessment is often viewed as best practice as a part of formal evaluation procedures especially when working with multilingual children, language and modality are typically assessed consistent with language majority (i.e., spoken Standardized American English) where the AAC user has the possibility to be scored as deficient, given that their language input does not match their output, and the full range of their languaging practices is not always accounted for (Hunt et al., 2022).

Through the IEP process, “setting decisions often reflects centering one need over the other: disability *or* language”, with students mostly being placed in special education classrooms with pull-out speech therapy (Cioè-Peña, 2022, p. 801). Typically, the way these services are delivered in the classroom are such that classroom teachers emphasize the use of a form of communication that they can understand themselves (i.e., oral/aural communication), paired with visual supports (e.g., visual schedules, graphic symbols) within the confines of structured lessons (e.g., circle time, activity). Unstructured communication interactions are neglected as classroom teachers are left without a systematic way of providing language support. However, students who use AAC are often prescribed specialized academic instruction in addition to specialized language and communication services supported by the speech therapist. Direct service delivery models (e.g., speech therapy twice weekly in an office setting, outside the classroom), alone, and current common practices on many public-school campuses cannot meet the needs of multimodal language users.

The Need for a New Framework

Speech-language pathology, as a field, pathologizes language and disability as it closely adheres to the medical model of disability (Staley et al., 2022). The foundation of speech-language pathology as a field is in the name: *pathology*. The Communication Sciences and Disorders (CSD) course curriculum teaches speech therapists to assess and view as deficient or disordered any individual seen as having speech, language, and communication skills that do not closely adhere to White, middle class, nondisabled, heteronormative ways of being, including the use of “Standardized American English” (Nair et al., 2023; Yu et al., 2021).

The Rehabilitation Act of 1973 and the Education for All Handicapped Children Act of 1975, which required school districts to provide a free and appropriate public education in the least restrictive environment, were pivotal in advancing the research and development of AAC devices. After the passing of each of these laws, AAC became an area of specialization, and the American Speech-Language-Hearing Association (ASHA) recognized AAC as a scope of practice within the field. An

Amendment to the Education of the Handicapped Act Amendments of 1986 (P.L. 99-457; Sass-Lehrer & Bodner-Johnson, 1989) law incorporated technological services for children with disabilities. Later, in 1989, the Technology-Related Assistance for Individuals with Disabilities Act (P.L.100-407) was passed, and this law required states to provide reasonable attempts to provide assistive technology to its residents (Medina, 2019). Despite legislation, speech therapists believed that the provision and use of an AAC device would inhibit speech and language development, an ableist viewpoint that persists among speech therapists and classroom educators today. Additionally, AAC supports and services are often provided by an AAC specialist who is usually a different speech therapist than the on-site speech therapist. This expert model of service delivery shifts the power and responsibility of AAC knowledge and implementation onto the AAC-SLP and allows for an attitude of ignorance around AAC implementation for AAC using students. These continue to be shortcomings in the service delivery and support of AAC among speech therapists and classroom educators in American public schools.

The aim of this article is to propose a novel conceptual framework for clinical practice, Translanguaging in AACtion, informed by AAC, translanguaging, and crip linguistics literature. Privette (2023) challenges us to “develop a praxis (theory and practice) that bends the arc of our profession toward justice,” including linguistic justice for disabled, non-speaking multimodal communicators. The purpose of the Translanguaging in AACtion framework is to challenge deficit perspectives toward multimodal communication and to push speech therapists and classroom teachers to apply educational justice and equity practices with modally marginalized individuals in clinical practice. The framework offers a systematic approach for supporting disabled, nonspeaking students to use their full, multimodal linguistic repertoire in every setting, with every person, in every situation. First, we offer a summary of the historical and contemporary context of AAC service delivery in public schools. Second, we define key terms that are integral to the Translanguaging in AACtion framework. Finally, we delineate the Translanguaging in AACtion framework into three elements: (a) service delivery, (b) reciprocal engagement, and (c) let them language. As you engage with this clinical focus article, we want you to keep in mind that this is an ongoing journey for each of us, and we want to invite you on this journey too.

Background

AAC

AAC refers to the different methods and systems that are used to communicate when a person does not have

consistent access to verbal, intelligible speech (ASHA, n.d.-a). AAC “is broadly thought of as a multimodal process involving various aided modes (e.g., traditional orthography, symbols, pictures, tablets with a communication application) and unaided (e.g. body language, signs, leading, gestures, actions, facial expressions, eye gaze, and vocalizations)” (Sutton et al., 2002, p. 192). Although the list of examples of unaided modalities is long, it is nonexhaustive and is provided to point out the way we use our bodies to do language, without external tools. Unaided forms allow their users to language freely without constraints and limitations connected to the availability of an external system. These modalities are used in combination to convey meaning and to supplement or replace spoken communication for individuals classified as speaking or nonspeaking. Aided AAC, commonly known as lite-, mid-, and high-tech forms, include both nonelectronic and electronic forms, respectively. Examples of nonelectronic forms (lite-tech) include pictures, graphic symbols, communication books and boards, and so forth. Electronic forms include battery-operated devices with a static display (mid-tech) and plug-in systems with a dynamic display and voice output capabilities (high-tech; Beukelman & Mirenda, 2013).

This categorization of AAC puts each languaging type into a neat package that encourages speech therapists to turn to a checklist of features that indicates what an AAC user is employing and not employing when communicating with others. If the AAC user is not languaging in the way that is expected, such as using their voice for spoken language, or even using their high-tech AAC system in a way that mirrors a spoken, named language (e.g., sequencing and selecting graphic symbols that place words in “correct” subject–verb–object grammatical word order or use a word in Standardized American English, which was not the targeted or expected response by their communication partner), they are labeled as “impaired.” However, it is important to remember that “We are all *language* users who use semiotic resources at our disposal in strategic ways to communicate and act in the world” (García & Wei, 2014, p. 10)—including those of us who do not use AAC as a primary communication form.

Representation and Barriers in AAC Systems

In 1991, the first ever communication aid that featured a touch screen with a dynamic display was shipped (Tobii Dynavox, n.d.). In this instance, the system was developed with a named language in mind. High-tech AAC system creators have historically focused on generating lexicons that can be used to reproduce the Standardized American English lexicon and grammar, namely, with subject–verb–object syntactic structures and

standardized orthography. Even when additional languages became available within the high-tech system, dialects (including their lexicons) of American English, such as AAE (Charity, 2008), did not and have not been represented or made available in the device. Also, the default programming for many AAC apps in the United States are for Standardized American English, and there is a significant amount of Standardized American English–sounding (Purnell et al., 1999) digital voices. It was not until 2021 (Lingraphica, 2021) that there was an on-demand way to change the skin tone of symbols to be representative of their racially and ethnically minoritized users. The first adult digital AAE voice, Tamira, became available in 2021; followed by Darius, the adult AAE digital male AAC voice; and Malik and Tiana, the first AAE digital AAC child voices in 2022 (Northwest Augmentative and Alternative Communication Society, 2021). When an individual receives a brand-new high-tech AAC system, the stock digital voice that comes on many of these high-tech AAC systems is the “Heather” voice, perceived to be an adult, White female. This feature must be manually changed upon receipt of the speech-generating device to reflect the user. Additionally, culturally appropriate preprogrammed graphic symbols are not always available, and operational features do not allow for system-wide customizations for phonology. Instead, to create changes to phonology within some high-tech AAC systems, one must edit graphic symbol by graphic symbol to replace or delete targeted sounds.

Historic and contemporary constructions of aided AAC tools have been built with a White, middle-class, heteronormative, “Standardized American English” language user in mind. These factors have pushed an already marginalized group—Black, disabled, nonspeaking, minoritized dialect users—further to the margins of accessibility for tools to communicate. As a result, many of these users have developed, honed, and relied on ways of languaging in their homes, communities, and with loved ones that are readily accepted and used, and have attempted to transfer them to academic institutions such as public schools, as legitimate ways of languaging. However, as observed by the authors and shared by families of multimodal language users, sometimes they have been met with misunderstanding, rejection, or discouragement by classroom teachers and speech therapists. Instead, students are often viewed as languageless and labeled as such because their ways of languaging are not always valued, considered, or accepted within the classroom. We point this out to remind us of the barriers that minoritized AAC users face when they turn to and rely on institutions to support or develop their communication skills. The skills they are taught are often not representative of them and can be limiting in terms of how they want to be viewed as multilingual and multimodal language users.

Grammaticality and AAC

Many speech therapists define language in a way that is consistent with ASHA: “Language is a rule-governed behavior. It is defined as the comprehension and/or use of a spoken (i.e., listening and speaking), written (i.e., reading and writing), and/or other communication symbol system (e.g., American Sign Language [ASL]); ASHA, n.d.-b). As speech therapists, we are taught theories of language development that emphasize the acquisition of vocabulary, morphology, phonology, and syntax of spoken language, happening in a very specific way (e.g., developmental norms). Theories of language development have historically excluded people who use nonspoken modalities as a primary communication form (e.g., Bates et al., 1996; Butler, 1974). AAC users are in a constant state of language contact between spoken languages, written languages, and symbolated languages. Researchers and speech therapists alike struggle to define and assess the language skills and abilities of AAC users. Accurately assessing the language skills of multimodal language users can be difficult (Nelson, 1992). The ability to quantify the language skills of AAC users (e.g., type-token ratio or mean length of utterance) is a concern that persists among some speech therapists in the field. This concern is rooted in the lack of clarity around how AAC users translate spoken input into symbolated output. Furthermore, it is unclear whether graphic symbol users construct a message based on the constraints of a spoken, named language, if there are constraints for graphic symbol production, or if AAC users translate spoken language into a graphic symbol. When whole message utterances are stored under a single keystroke, they can be difficult to analyze, because they are not constructed from smaller units, which is what is taught to speech therapists about traditional language analysis methods. However, within a translanguaging framework, we must ask ourselves if this type of characterization and description of language use is necessary. Instead, descriptions of language use by AAC users should focus on communication functions (e.g., greeting, commenting, requesting, describing, gossiping, sharing a story), use of aided and unaided forms and systems to communicate with others, and literacy. In the first author’s experience from a decade of clinical practice, AAC communicative success happens when students are understood sufficiently to get their wants, needs, and desires met and have their ideas conveyed, with minimal communication breakdowns between them and their communication partner.

The difficulty of understanding the structure of symbolated language is also impacted by the design of the system. As stated above, while there are key differences between software, an underlying feature across most AAC

systems is that they are built with Standardized American English grammar in mind, which emphasizes following American English orthography. However, AAC users may not follow the same grammar as spoken language nor should they be expected to. In the same way that ASL is not signed English but is often equated and conflated with Signed Exact English (SEE sign), graphic symbols in AAC devices should not be viewed as exact symbolized spoken English. Thinking of AAC as exact symbolated spoken English contributes to the language neglect that AAC users experience as their ways of languaging are counted as errors or viewed as being wrong. When language input does not match language output of AAC users, AAC users cannot be expected to conform to the expectations of their speaking communication partners, including their classroom teachers and speech therapists who do not consistently position themselves to perceive all of the ways that AAC users combine aided and unaided AAC, including graphic symbols, to share a message. The power differential between a communication partner that uses spoken language and a disabled, nonspeaking student who uses AAC places the onus on the AAC user to exist within the linguistic world of the speaking individual to make themselves understood. However, communication requires the attention of all parties involved (Ibrahim et al., 2023). Shifting our perspective on multimodal communication reminds speaking individuals of their responsibility to hear, see, feel, and respect the messages that multimodal language users convey, and it challenges classroom teachers and speech therapists to learn the language of their students and their families to then foster educational environments that fully embrace all ways of languaging.

Terminology

To provide a foundation for this framework, we will define some key terminology. To understand the need for the proposed framework, Translanguaging in AACtion, one must first understand that race, language, and culture are inextricably linked. The United States has a multigenerational legacy of language ideologies that prop up the pathologization of speakers who use language in a way that does not fit the White mainstream ways of being, which is perpetuated in speech therapy assessments, interventions, and practices, and in American public-school classrooms. The CSD profession adheres to a monoglossic ideology that pathologizes speakers who veer away from what is considered “normal” and label them as being “bad with language,” different, or deficient. We challenge you to reflect on what these labels mean. Can an individual really be “bad with language” (Henner & Robinson, 2023b, p. 7)? What would that look like? How would we

truly define that in a way that does not pathologize the language user? In practice, who and what are these individuals different from? The concepts below help us to think about these questions and inform our integration of the theories we discuss.

Raciolinguistic ideology is an analytical framework that originates from a field of study in which researchers examine how racialization and language interact (Flores & Rosa, 2015). Developed by Nelson Flores and Jonathan Rosa, raciolinguistic ideologies brings together critical race and critical language studies to illustrate the “co-naturalization” of race and language (Rosa & Flores, 2017, p. 621). Rosa and Flores (2017) point out that this co-naturalization occurs through and is the result of systemic inequity including “historical and contemporary colonial co-naturalizations of race and language; perceptions of racial and linguistic difference; regimentations of racial and linguistic categories; racial and linguistic intersections and assemblages; and contestations of racial and linguistic power formations” (p. 621). The raciolinguistic ideologies framework names the formation of normative categories as being the result of colonization and to dominate and oppress racialized groups. Thus, this framework has continued to have present-day consequences for those groups. In the current case study about Michael—the Black, disabled, nonspeaking, AAE-using, multimodal communicators within America. This framework challenges deficit perspectives of languaging in racially and linguistically minoritized groups by taking into account the historical, political, economic, and sociocultural structures that link race and language and consequently position minoritized communicators as languageless regardless of their actual language practices (Rosa, 2016). We use raciolinguistic ideologies to de-center White, middle-class, Standardized American English structures. In this clinical focus article, we will use raciolinguistic ideology to point to the intersection of race and language with an emphasis on how and why they cannot be separated, including within disabled, nonspeaking bodies.

Ableism is the practice of discriminating against disabled people in favor of abled bodyminds (Sins Invalid, 2019). “Ableism is an attitude or behaviour borne out of privilege of taking an action for granted” (Moulam, 2022). Ableism creates a binary between able-bodied and non-able-bodied people. In the field of speech pathology, “Traditional practices are based on identifying and remediating disability to help disabled people fit in with those who don’t have a disability” (Evenich, 2024). When we challenge this binary, we position ourselves to think about how one can participate in languaging without having to produce spoken language and can instead communicate effectively in other ways using their bodies and semiotics (Padia, 2023). Ableism is what leads speech therapists to

recommend that we “wait and see” if a child will develop verbal speech even when their prognosis for developing intelligible, verbal speech is considered “poor” based on their medical diagnosis. Ableism in speech therapy practice is also centering the use of a high-tech speech-generating device to be understood, while ignoring bids for communication using no, lite-tech, and mid-tech means.

Nonverbal and *minimally verbal* are long-used terms to describe individuals who do not have reliable access to their mouths to produce spoken language. This reliability can be impacted by anatomical or physiological disabilities, situational mutism, or other known and unknown factors (American Psychiatric Association, 2022). Oxford Languages (n.d.) defines *verbal* as “relating to or in the form of words.” Words can be expressed not just via spoken language but also through signed languages, written forms, pictures, signs, gestures, graphic symbols, actions, eye gaze, and more. Additionally, self-advocates and advocates for the AAC community have suggested that *nonverbal* refers to someone without words and perhaps alludes to having nothing to say (Goulbourne, 2018; Pruyn & Berg, 2021). However, nonspeaking individuals have words, even if they cannot be expressed through spoken language (Riggs, 2021). We use the term *nonspeaking* versus *nonverbal* as a stand-alone descriptor to discuss people who are not able to use their mouths to produce spoken language. Although *nonverbal* and *minimally verbal* are terms that persist among school-based professionals including classroom teachers and speech therapists, *nonspeaking* is a more appropriate descriptor of how the nonspeaking community uses and engages with language via multiple semiotic modalities and will be the term used by the authors to refer to people who primarily use AAC to communicate (Padia, 2023).

Semiotics was introduced to the field of speech-language pathology and AAC in 1993 by Gloria Soto and Wendy Olmstead (Soto & Olmstead, 1993). Semiotics is the study of the rules that control the generation, production, transmission, and use of signs, as well as the interchange, reception, and interpretation processes (Soto & Olmstead, 1993). A sign represents any referent, whether it is tangible or intangible, that has significance to the interpreter (Eco, 1984; Soto & Olmstead, 1993). Clarke (1990) identified five key semiotic positions, two of which we will share for our purposes: (a) signs are either natural or conventional and (b) sensory images count as natural signs that have iconic representation. Semiology considers communication to be much larger than spoken, named languages and acknowledges other signs—including non-verbal signs such as gestures, actions, signed languages, written forms, symbols, pictures, and so forth—in communication transmission and interpretation processes. For the purposes of this clinical focus article, the authors will refer

to semiotics to discuss the many modalities and combinations thereof.

Multimodal communication describes the many ways that people engage with their body and external tools and systems to communicate with others (Henner & Robinson, 2023b). With this definition, language and communication is not just spoken. For example, a person may use facial expressions, leading, turning to walk away, graphic symbols, gestures, symbols, pictures, high-tech communication system on a tablet, and much more. This list of ways that people may communicate is nonexhaustive, which illustrates the importance of not minimizing or reducing the creativity of language and communication by its users. The authors use the phrase “multimodal communication” to encompass the varying ways that a person may use language.

The authors use the term *multilingual* to refer to individuals who use more than one language to communicate. Historically, the terms *bilingual* and *multilingual* have been descriptors used to refer to people who use multiple spoken, named languages (e.g., Cenoz, 2013; Kemp, 2009). However, individuals who use a signed language in addition to a written language, for example, are also multilingual. AAC users who are raised in monolingual-speaking homes, schools, and communities are also often viewed as being monolingual. However, nonspeaking AAC users are expected to exist in settings that provide aural–oral input while they must respond using nonspeaking communication forms. As a symbolated system that relies on visual forms—often in the form of graphic symbols or pictures—AAC constitutes a language. Thus, we expand the traditional definition of multilingual to include AAC users and proceed on the basis that people who are multilingual have a large linguistic repertoire that is made up of more than one language and modality. With this background in mind, the next section summarizes the two scholarly disciplines that inform the Translanguaging in AAC-tion framework: Translanguaging and Crip Linguistics.

Theoretical Foundation

Translanguaging

Translanguaging is a theory of language that refers to the ways in which multilingual individuals access and use their full linguistic repertoire when communicating. Translanguaging centers the languaging experiences and practices of multimodal language users as multilingual language users as it recognizes that “all translanguaging is multimodal” (García & Wei, 2014, p. 40). The theory suggests that instead of having two (or more) distinct language systems, multilinguals have one system that houses

all of their meaning making tools. Additionally, in order for learners to be able to fully access their learning experiences in a way that can showcase all of what they know, they must have full access to their entire linguistic repertoire to make meaning and demonstrate understanding (Otheguy et al., 2015).

Although translanguaging focuses on and centers multilingual language users who use a spoken language, the current limitation of the translanguaging theory is that it focuses primarily on spoken, named languages generated by speaking, nondisabled communities rather than the symbolated communication systems used by nonspeaking, disabled communities. The Translanguaging in AAC-tion framework, then, expands translanguaging to center racially, linguistically, and modally minoritized disabled and nonspeaking communities.

Crip Linguistics

Crip linguistics, a theory proposed by Henner and Robinson (2023b), is born of a desire to reimagine the ways in which society perceives and receives disabled people’s ways of doing language. Crip linguistics comes from deaf education research and brings together translanguaging, raciolinguistics, and language deprivation to posit that language and disability are defined by normative standards and thus deficit perspectives “disorders language” rather than language being inherently impaired (Henner & Robinson, 2023a, p. 1). They go on to explain that “to crip language is to embrace disabled ways of languaging” (Henner & Robinson, 2023a, p. 2). Crip linguistics tells us that we do not need spoken language to communicate effectively. Instead, we must shift our praxis to lend itself to understanding that any meaning making through a combination of semiological resources that equalizes all the modalities needed to use language is a sufficient and valid way of languaging. While crip linguistics centers multimodal communicators with an emphasis on deaf language users, we use this as a launching point to center individuals who are nonspeaking as a result of a congenital, acquired, or situational motor speech impairment.

Toward an Expansive Approach

Verbal Speech Does Not Equal Language

Disabled and nonspeaking students of color are often viewed as languageless. Rosa (2019) coined the term *languagelessness* to “characterize simultaneous stigmatization of U.S. Latinx’s English and Spanish linguistic practices ... as deficient regardless of the extent to which they

might be perceived as corresponding to standardized norms.” Here, the authors use the raciolinguistic ideology of languagelessness to refer to the ways Black, nonspeaking multimodal language users living in AAE-speaking communities are pathologized as having no language. At school, and without the tools or devices prescribed by institutions, they are viewed as languageless both because they use nonspoken language as a primary form of communication and because they belong to an AAE-speaking community. They have been labeled as having no foundational language, specifically because as they exist, they are not seen as being able to participate in traditional curriculum and schoolwide assessments state and national standardized assessments to measure their “intelligence.” Educational professionals, including teachers, administrators, and speech therapists, operate as agents of the institution. Thus, we challenge you to consider whether a nonspeaking student can actually have no language. We believe that everyone languages; however, nonspeaking individuals are often characterized (read: dehumanized) as lacking language. When talking about nonspeaking AAC users, deficit descriptions are often employed to characterize them, such as “they don’t talk,” “I’ve never heard them speak,” and “they just sit there,” instead of descriptions that identify their language practices, such as “they primarily use gestures and a communication board to communicate” or “they use a speech generating device to chat with others.” This deficit framing can be tied to ableism and the historic, very difficult process of obtaining a robust high-tech AAC system, which was costly in terms of time, money, and resources, and therefore was often reserved for the privileged elite.

Speech Opportunities Over Language Deprivation?

Language acquisition and development are impacted by institutional and social factors (Hall et al., 2017). Language deprivation is a concept that originated from deaf education to refer to the lack of full access to a natural language during early language development in the critical first 5 years of life (Newport, 1990). Historically, D/deaf children were placed in oral-only language programs and schools to teach them to use spoken language at the expense of developing a language that was accessible to them (e.g., American Sign Language). Years of research within the deaf community has illustrated that exposure to and participation in a full, accessible language “has an independent influence on brain development separate from only the auditory experience of hearing loss” (Hall et al., 2017). Henner and Robinson (2023a) assert that “the consequence of structural ableism is language deprivation and the assumption that deaf children are languageless, without linguistic toolkits” (p. 5). We argue that the same is true for disabled, nonspeaking students.

As with deaf children, minimizing or reducing access to language expression in nonspeaking children with hopes that they will learn to use their voices to produce intelligible spoken language may have long-lasting consequences to their language acquisition and development. If emphasis is on spoken language, nonspeaking AAC users are not learning and practicing their multimodal language use. If we prioritize spoken language, we are robbing them of these faculties and capacities early on. Children who use AAC systems often experience language deprivation of the symbolized system and do not have linguistic capital as this language deprivation is driven by negative attitudes toward AAC. In education and clinical practice, the first author, Karina, often hears that professionals and parents want to delay the start of implementing formalized AAC tools, strategies, and supports until children are 5 years old, or at least in kindergarten, which is arbitrary. However, this delay contributes to language deprivation if in fact the nonspeaking individual is a multimodal and symbolized language user as their primary language form. If an individual cannot use their mouths to produce spoken language, then they are using AAC.

We all use multiple modalities to communicate daily. For example, speaking individuals use a variety of semiotic resources such as emojis, hand gestures, facial expressions, paired with spoken or written discourse. Non-verbal communication forms including gestures, actions, and eye gaze are easily accepted and consistently used by speaking communities as a single modality or in conjunction with verbal speech to aid in the transmission of their message. However, these modes are often pathologized and viewed as illegitimate when nonspeaking communities rely on them either independently or in combination with other modalities. Nonspeaking children who use AAC are consistently immersed in environments where spoken language is used and prioritized. As a result, they hear and receive speech input directly and ambiently but rarely observe graphic and symbolized systems and other AAC being utilized and prioritized outside of structured lessons delivered in structured settings.

Understanding language deprivation as it applies to AAC users requires that we begin to accept that multimodal AAC users do not have the same opportunities to develop and practice their language as their speaking peers because their way of languaging is perpetually minimized, and they are continuously stripped of their agency and power in educational and communication contexts.

Agency, Power, and AAC

Within the context of schooling for young learners, Adair (2014) defines *agency* as “being able to influence and make decisions about what and how something is

learned in order to expand capabilities” (p. 219). Adair provides examples of young, typically developing children using their agency by helping choose unit topics, experimenting and engaging in open-ended questions and conversations, planning projects, supporting friends and classmates with ideas, and being able to explore text, materials, and resources in support of their own learning. Overall, Adair refers to student agency as children having more control over their learning opportunities and experiences. We must ask ourselves, what does agency look like for AAC users? Who gets to decide how, when, and with what tools nonspeaking individuals can use to express themselves and participate in the full range of communication functions to engage their learning environment?

Choice, even uncoerced, in labeling the blue cup as ‘blue’ with spoken language, sign language, or a high-tech AAC device does not constitute agency. Agency additionally requires the opportunity to use those same tools to label incorrectly, to refuse the labeling task entirely (with the associated consequences), or to ask for more difficult work. (Donaldson et al., 2023, p. 141)

Thus, agency depends on the actions (and inaction) of communication partners within the environment. Agency cannot be “straight-forwardly a personal capacity or attribute” (Valentine, 2011, p. 356) that a student may possess based on their disability, language, grade, age, race, or ethnicity (Donaldson et al., 2023).

Nonspeaking individuals are often identified as being “behind” their peers when they miss their first speech and language milestone. Children are expected to say their first word by 1 year of age and be able to say at least 50 words by 2 years of age (National Institute on Deafness and Other Communication Disorders, 2022). When children are not using spoken language according to these milestones, they are often referred for an evaluation with a speech therapist. Sometimes, this evaluation includes AAC, at which time children are fitted for lite-, mid-, or high-tech systems. Especially within high-tech speech-generating devices, choices about the content of the AAC system is prescribed and controlled by the assessing speech therapist. “The content of AAC systems, the concepts and ideas included within the vocabulary available for young children is typically not under their control because these systems are designed by speech therapists or teachers” (Sutton et al., 2002, p. 193). In these moments, nonspeaking children do not have the power or agency to make changes to their vocabularies within their assigned speech-generating devices because the systems are often password protected and typically the only person who

holds the password is the speech therapist. Despite speech therapists and classroom teachers asserting that they use a total communication approach to communicate with these students, well-meaning classroom teachers direct and demand high-tech AAC users to use their tablet to request an item after they have expressed a request for that item via other modalities: sign, graphic symbol, gesture, or word approximation. Nonspeaking students having communication agency means that they are encouraged to use the full range of their semiotic resources to communicate and that those are accepted by their communication partner, without resistance or redirection to communicate in a different modality. AAC users having agency in their communication allows for the possibility that they can take full advantage of their multimodal, linguistic repertoire to support their learning, socialization, and recreational experiences and endeavors. When we hamper their agency, we strip their power—their power to communicate, learn, and language in the ways that they deem as efficient and effective to get their needs met.

Translanguaging in AACtion

Let’s revisit Michael’s story. Heeding his parents’ concerns, Michael’s educational team eventually referred him for a comprehensive AAC evaluation to be conducted by an AAC-SLP. The AAC-SLP recommended a total communication approach and ordered a speech-generating device with a dynamic display. The school team was excited to receive the system, so the school speech therapist set it up and gave the device directly to the family. Michael’s dad contacted the AAC-SLP to inquire about the device. He imperatively stated, “Why does it sound like that!?” After a series of questions and a long conversation, the dad was upset that the voice on the device did not sound like his son and that the graphic symbols within it did not reflect the language of his family and community (i.e., AAE) and Michael’s way of being. After a few weeks, his classroom teachers reported that Michael began hiding, turning off, and pushing the device away at school. His parents noticed the same thing at home. Upon further investigation, Michael’s established ways of languaging (e.g., signs, gestures, eye gaze) were being suppressed in favor of him learning to use the SGD, and communication was consistently being demanded from him (e.g., “say_____”; “ask for it with your talker”) in a way that it had never been before, even if he was clearly understood the first time around. The family continued feeling unseen and unheard by the “experts” and institutions meant to support them. What the family and educational team were looking for was *Translanguaging in AACtion*. Below, we outline the three elements of our Translanguaging in AACtion framework: (a) service

delivery, (b) reciprocal engagement, and (c) let them language.

Service Delivery

Service delivery models and dosage are a key component of IEPs and are considered based on data from the speech and language assessment results (ASHA, n.d.-c). The authors call on speech therapists to rethink their service delivery. Schmitt et al. (2017) found that only 15% of students receive the amount of service specified in their IEP. They further found that children's language severity does not predict treatment intensity and that more intensive language treatment, measured as time, is not significantly associated with improved outcomes (Schmitt et al., 2017). This means that in order to support students with their goals around improving their language skills, speech therapists need to be creative about how they deliver effective services. One way to do this is to provide classroom-based intervention alongside the classroom teachers to support teachers with incorporating strategies that encourage students to language authentically, using all of the modes, languages, and dialects that are a part of their repertoire. Speech therapists can model the practice and understanding that "good communication requires working with their students to understand them and to be understood" (Henner & Robinson, 2023a, p. 4). Speech therapists and classroom teachers, then, must position themselves to learn how to comprehend their students' language and expand that language rather than constraining it. Henner and Robinson (2023a) remind us that "in a just classroom, the language that the students use and their knowledge of it, regardless of any perceived fluency, is valuable. Classroom teachers [and speech therapists] need to have the resources to learn, learn well, and create an environment that students and teachers can build off each other's language" (p. 11). Speech therapists can help build and support just classrooms by fostering the use of a full-bodied semiotics by everyone in the classroom.

Reciprocal Engagement

Padía (2023) presents the idea of reciprocal carry-over. Reciprocal carryover focuses on encouraging classroom teachers to follow the lead of students and families as experts in their co-constructed communication and languaging systems, created and realized in support of their disabled and nonspeaking family member. Padía offers strategies to classroom teachers, families, and children on how to center the family and child as experts to co-construct communication and implement strategies from the home, in the classroom. These strategies include ethnographic interviewing, family observations, multidirectional communication logs, consent and dissent protocols

for nonspeaking, disabled students, professionals as co-conspirators to families, and infusing universal design for learning (UDL) with translanguaging practices (TrUDL; Cioè-Peña, 2022).

We recommend that speech therapists treat the delivery of therapeutic speech and language services as an opportunity to act as a mediator between families, communities, and classroom teachers. This would allow speech therapists to engage with families about their home and community language practices, share, and model those in the classroom and on the school campus with a variety of school professionals (e.g., teachers, administrators, para-professionals, speech therapists, and other practitioners). This also looks like inviting families and caregivers into the classrooms, the educational process, and school campuses and activities (e.g., fieldtrips, rallies) so that classroom teachers and speech therapists can observe how children and their families language in real time in a seminat-ural setting. These observations will inform key stakeholders on campus of the ways in which the families employ language practices to support, express, and understand each other outside school. It will also allow families to observe the languaging and communication that happen during structured lessons and educational practices at school and allow them to better support these endeavors at home.

Reciprocal engagement takes this one step further to put each of the strategies offered by Padía (2023) into AACtion. We call speech therapists and classroom teachers to offer the same level of communication engagement that they use with their speaking students to their nonspeaking students. Reciprocal engagement challenges the speaking communication partner to share in the work of communicating with their nonspeaking partner rather than expecting the nonspeaking individual to approximate spoken language before the communicative act is accepted as comprehensible. Disabled and nonspeaking children are often the ones who bear the most demanding role in communicating their wants and needs. However, "good communication requires a mutual respect of all available language and semiotic resources" (Henner & Robinson, 2023a, p. 4). Reciprocal engagement takes the onus from the nonspeaking student and asks that the speaking classroom teacher and speech therapist share in the work of communication by recognizing and embracing their own full-bodied semiotic expression, which involves expanding their own use of multimodal resources as well as their reception of their students' multimodal and multilingual communication strategies.

In real time, this looks like speech therapists and classroom teachers practicing patience and using wait time to truly engage with nonspeaking students. Johnson and

Parker (2013) found that implementing 5-, 10-, and 15-s wait times were effective in supporting children with sensory and multiple disabilities while communicating in their home. We urge speech therapists and classroom teachers to “crip time” with their disabled, nonspeaking students by challenging the imposed urgency of ableist normativity and transforming the communication environment rather than the communication partner (Ljuslinder et al., 2020). This means slowing down to share in the communication process and making accommodations that decrease time pressure when communicating. Creating a schedule that minimizes urgency in movement through the schedule can support crippling time and encourage everyone to slow down enough to meaningfully language and reciprocally engage.

Let Them Language

Language is inherent and cannot be separated from its user. According to Henner and Robinson (2023a), “language, as an embodied action, cannot be separated from the bodies that are read as deficient or non-normative in some way” (p. 3). In traditional practice, AAC users are expected to be able to use AAC fluently and proficiently. AAC users are pathologized when SGDs are not used “correctly” and they are expected to use them in a specific way before instruction is provided. It has been observed and reported by students and families just like Michael’s that if they use Standardized American English grammar “out of order,” omit morphemes, do not construct phrases or sentences that are long enough, or use too many modalities in one conversation, they are told that they cannot be understood. If they are perceived to be off topic, disabled, nonspeaking students are promptly redirected with minimal room to share about their interests in an unstructured setting and spontaneous conversation. Disabled nonspeaking students are forced to adhere to the language standards and discussion topics of their communication partners; and if they do not adhere, they are labeled as impaired or disordered and in need of “fixing.”

In structured and unstructured settings, assessments, and classroom instruction, translanguaging allows for an increasingly accurate way to measure a student’s skills and abilities. Letting students language goes beyond named languages and allows students to use a full-bodied semiotics to form deeper connections with their families, classroom teachers, and peers. We use languaging practices to be seen, to be heard, and to connect with others. When we support and encourage students to access their full linguistic repertoire throughout the school day, we can create and sustain a safe space for learning and growing in educational, social, and recreational domains. Translanguaging with multimodal language users makes room for AAC users to have “the same opportunity that monolinguals have always had, the

opportunity to learn and grow while enjoying the intellectual and emotional benefits of all of one’s linguistic resources” (Otheguy et al., 2015, p. 305). AAC users should have the agency and autonomy to have all languaging options including languages, dialects, and modalities available to them at all times to influence and optimize their learning, communication, and identity development.

Conclusions

This clinical focus article focused on reframing how we think about language in the disabled nonspeaking community, especially as related to our roles as speech therapists and classroom teachers. The authors provided a framework, Translanguaging in AACtion, to provide a praxis around engaging with and supporting all language users in and beyond the classroom by rethinking service delivery, supporting reciprocal engagement, and letting them language. Putting translanguaging in AACtion “empowers both the learner and the teacher, transforms the power relations, and focuses the process of teaching and learning on making meaning, enhancing experience, and developing identity” for all learners, including those who are nonspeaking (Wei, 2018, p. 15).

Disabled nonspeaking multimodal language users, like Michael, are looking for champions in the languaging space. Raciolinguistic ideologies perpetuated by monomodalists have slowed the languaging and learning progress of racially minoritized AAC users living in linguistically minoritized communities. Practices that focus primarily on providing intervention in the areas of articulation and intelligibility over supporting language building and learning are rooted in ableism and have negatively impacted the languaging experiences of nonspeaking students. As multimodal communicators, nonspeaking students are forced to navigate communication interactions with their peers, families, classroom teachers, and community members, without full support of their ways of languaging. Children should be able to use a full-bodied semiotics (modes, languages, and dialects) so that they can reach the same level of engagement as their speaking communication partners. However, this requires that their communication partners accept their communication practices as valid so that they remain open to comprehending and reciprocating the message. This is language and communication in the way that every human deserves: onward.

Positionality Statements

Karina R. Saechao is a Black, nondisabled, cisgender woman, and fifth-generation Oakland, CA native.

Karina is a multilingual African American English and Mainstream American English speaker. Karina is a first-generation college student at all levels: bachelor's and master's degrees and now doctoral student. As a doctoral student, graduate teaching assistant, and speech therapist, Karina is a scholar–researcher–practitioner. Karina has been afforded many privileges including educational privileges that have situated her to closely observe and be a target of the oppressive attitudes and practices that surround being a speaker of a minoritized, stigmatized, and often co-opted language within institutional settings. Karina's work as a speech therapist, primarily serving both racialized and nonracialized, multilingual, disabled, nonspeaking students, has pushed her to situate her work (i.e., practice and research) within equity-focused, social justice frameworks.

Chelsea Privette is a Black, nondisabled, cisgender woman born and raised in the U.S. South. As a heritage speaker of Black Language, she is familiar with the stigma of speaking a minoritized language and the psychological impact of suppression to meet “academic standards”—not just through language, but through appearance, behavior, and values. Having studied Spanish throughout her schooling and throughout university, she has also experienced firsthand the assumptions classroom teachers and clients/families carry about Black people's (in)ability to be multilingual.

John Kim is a first-generation doctoral student, a clinical instructor, an augmentative and alternative communication specialist, and a speech-language pathologist in California. John identifies as a nondisabled one-and-a-half-generation Korean American who immigrated to the United States in the late 1990s. As an immigrant in the United States with a single mother, his lived experiences of navigating the intersecting identities have situated his research to equity and social justice centered service delivery to families and individuals who are adversely affected by the U.S. education system. Thus, John's research is dedicated in equity-centered, family-centered approaches for those who have complex communication needs.

Data Availability Statement

Data sharing and availability are not applicable to this clinical focus article. The case study included in this clinical focus article represents a composite of many students. No data were collected, and no data sets were generated nor analyzed.

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